

INVESTIGATING CULTURAL FACTORS INFLUENCING SEXUAL AND REPRODUCTIVE HEALTH RIGHTS (SRHR) SERVICE ACCESS AMONG ADOLESCENT GIRLS AND YOUNG WOMEN (AGYW), AGED 10–24, IN NAIROBI, KENYA

WG–4: INACCESSIBLE SEXUAL AND REPRODUCTIVE HEALTH RIGHTS SERVICES FOR ADOLESCENT GIRLS AND YOUNG WOMEN

CHAPTER ONE

INTRODUCTION

1.1 Background

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The World Health Organization (WHO) defines young people as individuals aged 10–24, a demographic acutely vulnerable to sexual and reproductive health and rights (SRHR) crises (WHO, 2024). Globally, nearly 12 million girls aged 15–19 are expected to give birth in 2025, and complications from pregnancy and childbirth account for roughly one in every 23 adolescent deaths worldwide (UNICEF, 2025). Women and girls account for an estimated 44% of all new HIV infections globally (Murewanhema et al., 2025), and over 30,000 preventable reproductive deaths occur annually among adolescent girls and young women (AGYW), driven by poor health infrastructure, judgmental provider attitudes, and patriarchal norms that stigmatize contraceptive use (UNFPA, 2024; Guttmacher Institute, 2022; Melesse et al., 2021; WHO, 2023).

The burden of this crisis is not evenly distributed. Sub-Saharan Africa carries a disproportionate share of it: women and girls account for 62% of new HIV infections in the region, compared to 44% globally, and an estimated 3,100 AGYW acquire HIV every week regionwide (Murewanhema et al., 2025). The region's adolescent birth rate is more than double the global average, at over 100 births per 1,000 women in 2021 (Maharaj, 2022). Unequal power dynamics between AGYW and older partners

compound these risks, especially where poverty pushes young women toward transactional relationships for survival. Underlying many of these power imbalances are cultural expectations that normalize age-disparate relationships and discourage young women from negotiating safer sex, a cultural driver that this study examines directly.

Kenya mirrors this regional pattern while revealing its own internal inequalities. Nationally, teenage pregnancy declined from 18% in 2014 to 15% in 2022, yet this conceals disparities ranging from 50% in Samburu County to under 5% in Nyeri County (Kenya National Bureau of Statistics [KNBS], 2023). An estimated 330,000 adolescent girls become pregnant in Kenya every year (UNFPA Kenya, 2025). Youth aged 15 to 24 accounted for 41% of new HIV infections nationally in 2025, while pregnancies among girls aged 10 to 14 rose from 224,333 in 2024 to 235,938 in 2025 (Nation Media Group, 2026). Such wide county-level disparities are difficult to explain through health infrastructure alone, since counties with comparable resource constraints report very different outcomes; cultural variation in norms around early marriage, parental consent, and provider attitudes offers a more plausible explanation, and is the specific gap this study addresses.

Nairobi City County itself records a comparatively low aggregate teenage pregnancy rate of 8.4% (KNBS, 2023), masking sharp inequality within the city. In informal settlements such as Mathare and Kibera, AGYW face compounded poverty and limited health access; the median age of sexual debut among those surveyed was 16 years, with only 26% reporting condom use at last sex (Ziraba et al., 2018). The 2025 closure of DREAMS has withdrawn HIV outreach for roughly two million AGYW regionwide (UNAIDS, 2025).

1.2 Problem Statement

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Despite SRHR being recognized as a fundamental human right (Orlando et al., 2025; Sedgh et al., 2025), AGYW continues to face suboptimal access to SRHR services. Existing research attributes this largely to provider attitudes, structural barriers, and declining donor funding (Dzinamarira et al., 2026; Melesse et al., 2021). AGYW in Southern and Eastern Africa remain twice as likely to be living with HIV as their male counterparts, and barriers to access continue to drive unintended pregnancies, school dropout, and economic marginalization (Ayalew et al., 2022; UNFPA Rwanda, 2024). Much of this research has been conducted at national or regional level, drawing on health system data and large scale surveys, which makes it useful for identifying broad patterns but less able to explain why access remains so uneven between communities facing similar economic conditions (Ninsiima et al., 2021).

Much of the existing literature on SRHR access among AGYW in Sub-Saharan Africa, however, examines structural, economic, and health system barriers in general terms, without specifically interrogating the cultural factors that shape how AGYW perceive and pursue SRHR information and services (Ninsiima et al., 2021; Ouahid et al., 2025). Yet culture is a key factor that influences behaviour, including attitudes to accessing SRHR, and how it interacts with these structural barriers is not yet well understood, particularly within urban informal settlements, where AGYW face some of the region's highest rates of HIV infection, unintended pregnancy, and maternal mortality (Okumu et al., 2023; Chola et al., 2023). This gap has become more urgent following the loss of major donor funded programmes such as DREAMS, which has left a critical vacuum in how AGYW access reproductive health information and support, and about which little is currently known. It is also unclear what social or physical consequences AGYW themselves associate with seeking this information within their own families and communities, a dimension that is easily overlooked in studies that focus only on health system barriers. While cultural factors can in principle both enable and constrain SRHR access, this study focuses specifically on the barriers AGYW themselves identify, since these remain the least documented dimension in the literature reviewed above. This

study, therefore, seeks to fill that gap by investigating the cultural barriers to SRHR access among AGYW aged 10 to 24 in Nairobi, Kenya, including how these barriers interact with one another and shape AGYW's everyday health-seeking decisions.

1.3 Research Questions and Objectives of the Study

Authors: Lvy Oguye, Abubakar Diallo & Joy Chepkorir Bett

1.3.1 Objectives of the Study

General Objective

To investigate the cultural factors that influence access to sexual and reproductive health and rights (SRHR) services among adolescent girls and young women (AGYW) aged 10–24 in Nairobi, Kenya.

Specific Objectives

- To identify the cultural barriers that prevent AGYW aged 10–24 from accessing SRHR services in Nairobi, Kenya.
- To explore AGYW's perceptions of how cultural biases and moral policing by healthcare providers within clinics shape the youth-friendliness and accessibility of SRHR services in Nairobi, Kenya.
- To identify the social, familial, and physical risks that AGYW aged 10–24 associate with being discovered seeking SRHR services in Nairobi, Kenya.

1.3.2 Research Questions

RQ1: What cultural barriers prevent AGYW from accessing SRHR services in Nairobi, Kenya?

RQ2: In what ways do clinical environments and the personal biases of healthcare workers create a culturally unwelcoming environment for AGYW seeking SRHR services in Nairobi, Kenya?

RQ3: What social, familial, or physical risks do AGYW face within the community if they are discovered seeking information about SRHR services in Nairobi, Kenya?

1.4 Relevance of the Study

Authors: Lvy Oguye & Abubakar Diallo

Cultural factors, most notably patriarchal power dynamics that enable reproductive coercion within intimate partnerships, shape AGYW's decisions to seek SRHR services, contributing to unintended pregnancies, STIs, and unmet contraceptive needs (Ninsiima et al., 2021). With Sub-Saharan Africa hosting one of the world's largest youth populations, understanding these dynamics is critical for designing responsive, youth-friendly interventions. This study equips policymakers, healthcare providers, and educators with culturally sensitive strategies, aligning with SDGs 3 and 5 and the AU's Agenda 2063 (United Nations, 2015; African Union Commission, 2015). Concretely, programmes that fail to account for these cultural dynamics risk repeating the access gaps documented in Section 1.1 even where funding and facilities are adequate, which makes cultural insight a precondition for effective SRHR programming rather than an optional add-on.

1.5 Significance of the Study

Authors: Alungat Patience, Denis Njoroge & Joy Chepkorir

Employing feminist standpoint theory, this study positions AGYW as experts on their own circumstances, generating qualitative insights into how cultural barriers are

reproduced at community, family, and institutional levels, insights quantitative analyses cannot capture. This study's focus on Nairobi addresses a documented evidence gap: context-specific data from East Africa, a sub-region where AGYW face the world's highest rates of HIV infection, unintended pregnancy, and maternal mortality, remain scarce (Ouahid et al., 2025). Evidence confirms that cultural factors and intersecting stigmas shape how AGYW evaluate seeking care (Okumu et al., 2023; Chola et al., 2023). By centering AGYW's own accounts of how these cultural barriers operate, this study offers policymakers, healthcare providers, and researchers an evidence base for designing gender-transformative SRHR programmes and policies, grounded in feminist standpoint theory's commitment to treating marginalized groups as authoritative sources of knowledge about their own lives (see 1.4 for the corresponding policy alignment). In doing so, the study moves the conversation beyond documenting that access gaps exist toward explaining, in AGYW's own terms, why they persist, which is the level of detail programme designers need in order to act.

CHAPTER TWO

LITERATURE REVIEW

Introduction

This chapter presents a combined literature review addressing the first two research questions guiding this study: the cultural barriers that prevent AGYW from accessing SRHR services in Nairobi, Kenya, and the ways in which clinical environments and the personal biases of healthcare workers create a culturally unwelcoming environment for AGYW seeking these services. The review is organized around two themes. The first, the vicious cycle of reproductive coercion and socioeconomic marginalization, treats patriarchal control over reproductive decisions and economic dependency as mutually reinforcing forces that restrict AGYW's autonomy in Nairobi. The second, clinical environments and healthcare worker biases as barriers to culturally responsive

SRHR services, examines how provider attitudes and facility conditions extend these same cultural dynamics into the clinic itself. Read together, the two themes show that the barriers AGYW encounter in Nairobi do not stop at the household or the community; they persist into the health system that is meant to serve them. Neither theme, however, directly addresses what AGYW themselves anticipate would happen, socially, within the family, or physically, if they were discovered seeking SRHR services, which is the specific focus of the study's third research question and the gap the fieldwork that follows this review is positioned to close.

THE “VICIOUS CYCLE” OF REPRODUCTIVE COERCION AND SOCIOECONOMIC MARGINALIZATION (THE INTERSECTION)

Authors: Joy Chepkorir Bett, Abubakar Diallo, Lvy Oguye

Reproductive coercion (RC) is a form of gender-based violence that undermines the reproductive autonomy of individuals capable of becoming pregnant through behaviors such as contraceptive sabotage, pregnancy pressure, and interference with clinical access (Miller et al., 2010; Lévesque et al., 2025). In Nairobi, these behaviors do not occur in a vacuum. They are underwritten by cultural expectations that assign reproductive decision-making to male partners, parents, or community elders rather than to AGYW themselves, and by religious and social norms that treat premarital sexuality and contraceptive use among unmarried young women as improper (Kinaro et al., 2019; Langat et al., 2024). Lévesque et al. (2025) argue that RC is best understood across interpersonal, community, and societal levels rather than as an isolated act between partners, since providers who restrict contraceptive counselling and social norms that place the burden of fertility control on women both extend RC beyond the household and into the institutions AGYW rely on for care. This ecological view is borne out in Nairobi's own data: nearly one in five partnered AGYW report lifetime RC, and 93.2% report transactional elements within their relationships that further complicate their ability to negotiate contraceptive use (Decker et al., 2021).

Cultural stigma, in other words, does not simply discourage AGYW from seeking SRHR services. It actively shapes the terms on which their partners, families, and communities permit them to do so.

Socioeconomic marginalization intensifies these cultural dynamics rather than operating alongside them. Poverty and economic dependence push young women in Nairobi's informal settlements toward transactional relationships in which negotiating condom use or contraceptive access carries a direct financial cost, deepening the same power imbalance that enables coercion (Ninsiima et al., 2021; Omojemite, 2026). Regional evidence helps explain why: Dozier et al. (2025) found that women with more than a secondary education had substantially lower odds of experiencing RC than women with no formal schooling, identifying education as a significant protective structural factor, while Omojemite (2026) centers financial dependence and unemployment as the dominant predictors of what the study terms forced motherhood, arguing that economic dependency normalizes male authority over reproductive decisions. Raihen et al. (2023) reinforce this pattern, showing that patriarchal power within intimate partnerships most constrains the choices of women who are least able to exit the relationship economically. For AGYW in Nairobi's informal settlements, where school dropout and limited income opportunities are already common, this relationship compounds an existing vulnerability rather than introducing a new one.

This produces a feedback loop rather than a single point of harm. Boyce et al. (2020) and Wood et al. (2020), drawing on qualitative work with women and girls in Nairobi's family planning clinics, describe partners who pressure AGYW into pregnancy or deliberately damage their contraceptive supplies, prompting many women to conceal contraceptive use altogether as a short-term safety strategy. In Nairobi specifically, 15.6% of partnered AGYW report non-consensual condom removal, and discovery of concealed contraceptive use frequently triggers partner violence and the destruction of clinical records, which blocks future access to SRHR services entirely (Decker et al., 2021). The unintended pregnancies that follow this cycle routinely end schooling

or vocational training, deepening the same financial dependence that made the original coercion possible, so that socioeconomic marginalization and reproductive coercion in Nairobi reinforce one another over time rather than resolving after a single incident (Ayalew et al., 2022).

Some evidence suggests this cycle can be interrupted, at least at the clinical level. A cluster-controlled trial of the ARCHES intervention across six private Nairobi clinics found that training providers to offer private, partner-separated counselling, including contraceptive methods that are harder for a partner to detect, produced a significantly greater reduction in attitudes justifying reproductive coercion relative to standard care by six-month follow-up (Uysal et al., 2023). Community health worker training piloted in Malawi offers a comparable model, shifting CHWs from judgmental gatekeepers into non-judgmental mentors who support adolescent mothers' social and educational reintegration (African Population and Health Research Center & Centre for Social Research, University of Malawi, 2021; Otukpa et al., 2025). Because this model was developed for adolescent mothers outside Kenya, however, it remains unclear whether it would address the specific clinical access barriers documented among AGYW in Nairobi's informal settlements, a question the ARCHES trial's Nairobi-based results only partially answer since the two interventions test different points in the same cycle.

CLINICAL ENVIRONMENTS AND HEALTHCARE WORKER BIASES AS BARRIERS TO CULTURALLY RESPONSIVE SEXUAL AND REPRODUCTIVE HEALTH SERVICES FOR ADOLESCENT GIRLS AND YOUNG WOMEN

Authors: Collins Bebeth , Denis Njoroge

Even where AGYW in Nairobi overcome the cultural and economic barriers described above, the health facilities they eventually reach frequently reproduce the same judgment they anticipated from their communities. Provider bias occurs when healthcare workers allow personal beliefs, values, or assumptions to shape the services they provide instead of relying solely on professional standards (Solo & Festin, 2019).

In Kenya, Godia et al. (2013) found that some healthcare providers viewed adolescent sexual activity as a moral issue rather than a health issue, which made it difficult for providers serving Nairobi's youth to separate personal belief from professional responsibility. O'Malley et al. (2021) reported a similar pattern among providers serving AGYW, who expressed concern that offering contraceptives or HIV prevention services could be seen as encouraging sexual behavior. Taken together, this evidence indicates that providers in Nairobi's clinics often act as gatekeepers, determining adolescents' access to services based on perceived social acceptability rather than clinical need, a pattern that closely mirrors the community-level gatekeeping already described in the discussion of reproductive coercion above.

Beyond individual attitudes, the physical and social environment of Nairobi's health facilities shapes whether AGYW feel safe enough to attend at all. Corley et al. (2022) identify privacy, confidentiality, and respectful communication as essential components of youth-friendly SRHR services, noting that young women may avoid services altogether if they fear being recognized or exposed to neighbors or relatives, a risk that is heightened in the dense, closely networked informal settlements where much of Nairobi's AGYW population lives. Kenyan evidence from Godia et al. (2014) found that adolescents associated clinics with long queues, inadequate consultation spaces, overcrowded facilities, and uncomfortable interactions with staff, with some participants reporting that they felt watched or embarrassed during consultations. These experiences reduce trust and discourage future service use, an effect that compounding facility conditions and negative staff attitudes make more severe still (Chakupewa & Mmari, 2021). A significant limitation runs through this evidence, however: most of it draws from girls who already attended a Nairobi facility, while girls who never attended because they anticipated stigma or judgment are largely absent from the data, meaning the true scale of avoidance in the city may be considerably larger than current research shows.

Unwelcoming experiences in Nairobi's clinics are not solely the product of individual providers. They are also shaped by the broader health system within which those providers work. Sidamo et al. (2023) identify poor provider training, weak adolescent-focused systems, and institutional practices as barriers to youth-friendly care, while Chakupewa and Mmari (2021) argue that negative staff attitudes combined with poor facility conditions jointly discourage adolescents from seeking care. There is evidence that these problems are addressable within Kenya specifically: Mutea et al. (2023) found that combining provider training with practical changes, such as improved clinic organization and extended service hours, increased adolescents' comfort with SRHR services. Still, most intervention studies, including this one, measure whether service use increased in the short term rather than whether provider attitudes changed durably, and little evidence exists on whether improvements in Nairobi's clinics persist once external support ends.

Recent literature increasingly frames provider bias in Nairobi as a reproductive rights issue rather than a service delivery issue alone. Senderowicz (2020) introduces the concept of contraceptive autonomy, the principle that individuals should be able to make reproductive decisions freely and without pressure, positioning healthcare workers as actors who can either support or restrict young women's choices. Tavrow (2026), writing on government health facilities in rural Kenya, extends this by describing healthcare workers as gatekeepers whose attitudes determine whether young people feel accepted and are willing to return for future services, a dynamic that plausibly holds with even greater force in Nairobi's higher-volume public facilities. Provider bias, in this framing, is not an isolated professional failing but an extension of the same cultural and religious norms that stigmatize adolescent sexuality in the wider Nairobi community (Sidamo et al., 2023), which is precisely why the vicious cycle described in the first theme of this review does not end at the household. It continues, largely unbroken, into the clinic.

Research Gaps

Several gaps recur across both bodies of literature reviewed in this chapter. First, the majority of quantitative studies on reproductive coercion in Nairobi, including Decker et al. (2021) and Wood et al. (2023), rely on cross-sectional designs, which establish that socioeconomic marginalization and RC co-occur but cannot confirm which comes first or whether the cycle intensifies the longer it continues. Longitudinal research tracking AGYW through this cycle over time is largely absent from Nairobi-specific literature.

Second, existing evidence on both reproductive coercion and clinical bias is concentrated among AGYW who already attend clinics or who are already partnered, leaving two related populations understudied: AGYW who avoid care altogether because they anticipate stigma or judgment, and unmarried or out-of-school AGYW whose experiences may differ from the clinic-attending samples most studies draw on (Ninsiima et al., 2021; Godia et al., 2014).

Third, the literature overwhelmingly centers on women's own accounts and, within clinical settings, providers' accounts, examined largely as separate bodies of evidence. Raihen et al. (2023) and Omojemite (2026) note the near total absence of research into male partners' or parents' motivations, community norms that enable coercion, or male engagement as a prevention strategy, while Godia et al. (2013, 2014) study providers and adolescents as separate populations rather than examining how the two groups negotiate cultural expectations together during an actual clinical encounter.

Fourth, while Lévesque et al. (2025) provide a useful ecological framework for understanding RC, and Mutea et al. (2023) demonstrate that combined provider training and structural change can improve service use in Kenya in the short term, neither line of research has been extended into evidence on whether these gains persist once external support ends, or whether the ecological framework itself has been empirically validated for Nairobi's specific institutional and cultural structures.

Finally, and most directly relevant to this study, no study reviewed here asks AGYW what they themselves anticipate would happen, socially, within the family, or physically, if they were discovered seeking SRHR services in Nairobi. The literature on reproductive coercion documents what happens after discovery within intimate partnerships, and the literature on clinical bias documents what AGYW experience inside the facility, but neither addresses the broader social and familial consequences AGYW associate with the act of seeking care itself. This is precisely the third research question this study is designed to answer, and it is the gap that the qualitative, descriptive phenomenological fieldwork described in Chapter Three is positioned to close.

Conclusion

The literature reviewed in this chapter shows that the cultural barriers preventing AGYW from accessing SRHR services in Nairobi operate as a single connected system rather than as separate obstacles. Reproductive coercion and socioeconomic marginalization reinforce one another at the level of the household and the community, while provider bias and unwelcoming clinical environments extend the same cultural norms into the health facilities AGYW eventually reach. Neither theme, on its own, resolves the barriers AGYW face; a woman who negotiates her way past a coercive partner may still meet a judgmental provider on the same visit, and a clinic that trains its providers well cannot, on its own, address the economic dependency that makes coercion possible at home. What the evidence does not yet capture, in Nairobi specifically, is longitudinal data on how this cycle develops over time, the experiences of AGYW who avoid care entirely, and, most importantly for this study, what AGYW themselves anticipate would happen within their families and communities if their efforts to seek SRHR services were discovered. Addressing this last gap through AGYW's own accounts, gathered directly from Nairobi's informal settlements, is the specific contribution the fieldwork that follows this review is designed to make.

CHAPTER THREE

METHODOLOGY

3.1 Introduction to the Methodology

This section presents the methodology adopted for this study. It outlines the research approach and design, the study area and population, the sample and the procedure used to select participants, the methods and tools used to collect data, and the steps taken to manage, analyze, and safeguard the quality of the data. It closes with a discussion of the ethical considerations relevant to working with adolescent girls and young women (AGYW) on a sensitive topic such as sexual and reproductive health and rights (SRHR). In keeping with the qualitative, experience-centred orientation of this study (see Significance of the Study), a qualitative approach using a descriptive phenomenological design is adopted, with the aim of describing and understanding the cultural barriers, sources of support, and risks that AGYW associate with accessing SRHR services in Nairobi, Kenya.

3.2 Research Design

3.2.1 Research Approach

This study adopts a qualitative research approach. Qualitative research is concerned with understanding the meaning people attach to their experiences and with exploring a phenomenon in depth, rather than measuring it numerically (Creswell & Creswell, 2018). This approach is appropriate for the present study because its guiding objectives and research questions are framed around “what” questions, namely what cultural barriers AGYW face, what sources they rely on, and what risks they associate with seeking SRHR services. These questions seek to explore meaning and lived experience rather than to measure frequency or test a hypothesis, which places the

study firmly within the qualitative tradition. This choice is also consistent with the feminist standpoint lens adopted by the study, which treats AGYW as experts on their own circumstances and prioritizes their own accounts over externally imposed measures (see Significance of the Study).

3.2.2 Research Design

The study adopts a descriptive phenomenological design. Descriptive research is designed to provide an accurate account of the characteristics of a particular situation, population, or phenomenon as it currently exists (Kothari, 2004). A descriptive design is appropriate here because the study seeks to describe, rather than to causally explain or statistically compare, the cultural barriers, support sources, and risks that AGYW in Nairobi encounter. Phenomenological research, in turn, is used when the researcher seeks to understand the lived experiences of people navigating a particular phenomenon, often one that is sensitive or stigmatized in nature (Creswell, 2013). This study draws on phenomenology because it is not enough to list barriers and risks; it also seeks to understand the meaning AGYW themselves attach to these experiences. Combining the two, the study is positioned as descriptive phenomenological: it describes the situation of AGYW accessing SRHR services in Nairobi while also capturing the deeper meaning participants attach to those experiences.

3.3 Study Area

This study will be conducted in Buruburu, a middle-income residential estate within Nairobi County, Kenya. Existing SRHR research on AGYW in Nairobi has concentrated heavily on informal settlements, where compounded poverty and documented vulnerability to early sexual debut and inconsistent contraceptive use have attracted sustained scholarly attention (Ziraba et al., 2018). Beguy et al. (2009), drawing on comparative data from slum and non-slum settlements in Nairobi, similarly observe

that adolescent sexual behaviour research has focused disproportionately on informal settlements, leaving non-slum urban communities comparatively understudied. Buruburu is therefore an appropriate study area precisely because it falls outside this concentration of existing evidence: cultural barriers, information sources, and risks that AGYW associate with seeking SRHR services in a middle-income urban estate remain largely undocumented, even though the same underlying cultural dynamics, patriarchal norms, provider bias, and community-level stigma, are not confined to informal settlements alone. Studying Buruburu allows this research to test whether the cultural barriers identified in the informal-settlement literature extend to a different socioeconomic context, or whether they operate differently where poverty is less acute but cultural and religious norms remain influential. Practically, the study will reach AGYW in Buruburu through two community entry points: adolescent participants identified with parental awareness through estate-based family and youth networks, and older AGYW reached through a salon in Buruburu frequented by young women in the target age range. These same entry points also provide the foundation for the two snowball sampling chains described in Section 3.6.

3.4 Study Population

The study population is AGYW aged 10–24 years residing in Buruburu, Nairobi. This population includes both in-school and out-of-school AGYW, since cultural barriers to SRHR access often differ between the two groups: in-school AGYW may encounter school-based gatekeeping or curriculum-related gaps, while out-of-school AGYW may face barriers rooted more in family expectations or economic dependency. Including both groups allows the study to capture a fuller range of cultural barriers, support sources, and risks than a narrower population would. This population is appropriate for the study because its objectives are specifically concerned with AGYW as a group, and because the age range of 10 to 24 reflects the WHO definition of young people used throughout this research (WHO, 2024, as cited in the Background). Given the study's five-day data collection timeline, the study population is limited to

AGYW directly; key informant perspectives from healthcare providers and community gatekeepers are not included in this round of data collection, and Objective 2 of this study is addressed instead through AGYW's own accounts of their interactions with clinical environments and healthcare providers, consistent with the study's overall focus on AGYW as experts on their own experience.

3.5 Study Sample

The study will recruit a total of 18 participants, drawn from the study population described above. This includes 12 participants for individual in-depth interviews and 6 participants for one focus group discussion, both with AGYW. In-depth interviews are expected to last approximately 45 to 60 minutes each, and the focus group discussion approximately 60 to 90 minutes. There is no overlap between the two groups.

Method	Number of Participants
In-depth interviews(IDIs) with AGYW	12
Focus group discussion (FGD) with AGYW	6
Total	18

This sample size is appropriate for a qualitative study of this kind, where the goal is depth of understanding rather than statistical generalizability. Twelve in-depth interviews are sufficient to reach a reasonable depth of insight into individual experiences of cultural barriers and risk, while a single focus group of six participants is included to capture shared community-level perceptions and norms that are more readily surfaced in a group setting than in one-on-one interviews. The split also reflects the sensitivity of the topic: individual interviews allow participants to disclose personal experiences of stigma or risk without the presence of peers, while the focus group is reserved for questions where shared, less personally identifying community

norms are the focus. Given the study's five-day data collection timeline, key informant interviews with healthcare providers and community gatekeepers are not included in this round of data collection; Objective 2 is instead addressed entirely through AGYW's own accounts of their experiences with clinical environments and healthcare providers, consistent with the AGYW-centred, feminist standpoint orientation of the study.

3.6 Sampling Procedure

AGYW participants will be selected using snowball sampling, a non-random procedure in which initial participants, identified through a community gatekeeper, assist in identifying further participants who meet the study's criteria. Snowball sampling is widely recommended for reaching populations who are difficult to identify through formal records and for studying sensitive topics where trust between the researcher and participant is essential (Naderifar et al., 2017). It is appropriate for this study for two reasons. First, AGYW who have direct experience of the cultural barriers and risks under investigation are not easily identified through formal lists or institutional records, particularly given the sensitivity of SRHR-related experiences; existing peer and trust networks are a more realistic route to reaching them. Second, because the topic involves disclosure of stigmatized or sensitive experiences, participants are more likely to take part, and to speak candidly, when approached through a trusted referral from someone they already know, rather than through a cold approach from an unfamiliar researcher. The study will draw on two separate seed-and-snowball chains to reach AGYW across the target age range in Buruburu. The first chain begins with adolescent participants identified through estate-based family and youth networks, with initial contact made with parents' or guardians' awareness from the outset; these seed participants then assist in identifying peers who meet the study's criteria. The second chain begins with older AGYW reached through a salon in Buruburu frequented by young women in the target age range, whose regular clients and their peers form the basis for further referrals. Using two chains, rather than one, is intended to reduce over-reliance on a single referral network and to help capture both in-school,

parent-connected adolescents and older, possibly out-of-school or working young women. The research team will monitor the diversity of the resulting sample, for example across in-school and out-of-school status and across the two chains, to avoid over-relying on either referral pathway. Recruitment will continue until the target of 12 IDI participants and 6 FGD participants is reached, or until thematic saturation is indicated by the absence of new themes across consecutive interviews.

3.7 Data Collection Methods and Tools

Data will be collected using a combination of individual in-depth interviews (IDIs) and one focus group discussion (FGD), both qualitative methods consistent with the study's overall approach.

3.7.1 In-Depth Interviews and Focus Group Discussion(AGYW)

In-depth interviews will be conducted with 12 AGYW participants using a semi-structured interview guide, and are expected to last approximately 45 to 60 minutes each. This format allows the interviewer to ask a consistent set of core questions across all participants while leaving room to follow up on points specific to each participant's experience, which is important given the personal nature of the topic. The guide is organized into thematic sections corresponding to the study's research questions, beginning with a biographical section before moving into the substantive themes, so that questions are grouped by topic rather than presented as a single undifferentiated list. A sample of the interview guide is included as Appendix A.

The focus group discussion will be conducted with 6 AGYW participants using a semi-structured FGD guide, organized into the same thematic sections as the IDI guide and following the same core questions framed for group discussion, with the addition of a probe on whether participants have heard of experiences differing for AGYW in other communities. This line of inquiry was raised during supervision but is retained at the level of an informal probe rather than a formal research objective, consistent with

the qualitative scope of this study. The FGD is expected to last approximately 60 to 90 minutes. The FGD guide is included as Appendix B.

Both the IDIs and the FGD will be conducted primarily in Kiswahili and Sheng, with English used where participants are more comfortable in English, reflecting the everyday language use of the study population in Buruburu. A bilingual member of the research team will conduct the sessions and will be responsible for translating and transcribing the recordings into English for analysis, with the original-language version retained alongside the translation so that meaning is not lost in the process.

All sessions across both methods will be audio recorded with participants' consent, in addition to handwritten notes taken by a note-taker present at the focus group discussion.

3.8 Data Management and Analysis

3.8.1 Data Management

Audio recordings from the interviews and focus group discussion will be transferred from the recording device to a password-protected laptop and a password-protected cloud storage folder as soon as possible after each session, after which the original recording on the device will be deleted. Each participant will be assigned a unique code (for example, P1 for the first AGYW interview participant) rather than being identified by name in any transcript, field note, or analysis document. Only members of the research team will have access to the raw recordings and transcripts; any data shared with supervisors or included in the final report will be anonymized.

3.8.2 Data Analysis

Data will be analyzed using thematic analysis, following Braun and Clarke's (2006) six-phase framework: familiarization with the data, generation of initial codes, searching for themes, reviewing themes, defining and naming themes, and producing the final write-up. This approach is appropriate for the study because it allows

patterns and themes to be identified across the IDI and FGD transcripts in relation to the study's objectives, namely cultural barriers, sources of support, and risks associated with seeking SRHR services, while remaining grounded in participants' own words and meanings rather than imposing a predetermined coding framework.

3.9 Data Quality Control

To ensure the trustworthiness of the findings, this study addresses the four quality criteria proposed by Lincoln and Guba (1985) for qualitative research:

- **Credibility:** After analysis, a summary of the emerging themes will be shared with a small number of participants who agreed to follow-up contact, to confirm that the themes reflect their actual experiences and perspectives (member checking). Data triangulation is also achieved across the two data sources collected, individual in-depth interviews and the focus group discussion, allowing individually disclosed experiences to be considered alongside shared, community-level perceptions of the same barriers.
- **Transferability:** The study area, population, and sampling procedure are described in sufficient detail in this section to allow readers to judge whether the findings might apply to AGYW in other middle-income urban communities with similar characteristics.
- **Dependability:** The research process, including the IDI and FGD guides, sampling procedure, and analysis steps, is documented clearly in this section and will be retained as a record so that another researcher could follow the same process.
- **Confirmability:** The research team will keep a brief reflexive journal during data collection and analysis, noting any personal assumptions or reactions, to help ensure that the themes that emerge are grounded in participants' data rather than in the researchers' own expectations.

3.10 Research Ethics

This study involves AGYW, including minors, discussing a sensitive topic, and therefore requires careful attention to ethical conduct throughout, in line with WHO (2023) guidance on engaging young people in health-related research. Given the inclusion of participants as young as 10 years, the research team will apply heightened safeguarding measures for participants under 16, including the presence of a trusted community adult during sessions and age-appropriate question framing. The IDI guide will be adapted to avoid questions that require participants to self-disclose specific personal experiences of abuse or risk.

Informed consent: All AGYW participants will be informed, in a language they understand, about the purpose of the study, what participation involves, the voluntary nature of participation, and their right to skip any question or stop at any time. Consent and assent are documented using separate, purpose-specific forms rather than embedded in this document: participants aged 18 to 24 will complete an adult consent form in their own right, while for participants under 18, written or verbal consent will be sought from a parent or guardian using a separate guardian consent form, in addition to the participant's own assent recorded on a separate, age-appropriate assent form, so that no minor is enrolled without both layers of agreement. Consent and assent will be sought in a private setting to reduce pressure to agree in front of others.

Confidentiality and anonymity: Participants will not be identified by name in any transcript, recording label, or written report; each will be assigned a code. Recordings and transcripts will be stored securely as described in the Data Management section above, and any quotations used in the final report will be anonymized and checked to ensure they cannot identify the speaker.

Right to withdraw: Participants will be informed, both at the start and during the session, that they may withdraw from the study at any point, including after the

interview or focus group has taken place, without any negative consequence, and without needing to give a reason.

Data protection: All recordings, notes, and transcripts will be stored on password-protected devices and cloud storage accessible only to the research team, and will be deleted from recording devices once securely transferred, as described above.

Do no harm: Given the sensitivity of the topic and the possibility that some questions may bring up distressing experiences, sessions will be conducted by trained members of the research team using a calm, non-judgmental approach, and will be paused or stopped if a participant becomes distressed. The research team will identify and share the contact details of a local psychosocial support or counselling service with all AGYW participants, regardless of whether they show signs of distress, so that support is available if needed after the session.

Institutional approval: No formal institutional ethics clearance has been sought for this study at this stage, as this is a bootcamp training exercise rather than a funded or published study. In place of formal clearance, the research team will seek explicit permission from the community gatekeepers, including parents or guardians and the salon owner facilitating access to participants, and will follow the consent and protection measures outlined above as the primary safeguard for participants.

Sample Data Collection Tools

A. Semi-Structured In-Depth Interview (IDI) Guide

Introduction (read aloud to the participant before starting): “Thank you for agreeing to speak with me today. My name is [Interviewer Name], and I am part of a research team looking into the experiences of young women like yourself in accessing sexual and reproductive health information and services here in Buruburu. There are no right

or wrong answers; I am simply interested in your own experience and views. You do not have to answer any question you do not want to, and you can stop at any time. Everything you share will be kept confidential and will not be linked back to your name. Do you have any questions before we begin?” [Obtain verbal consent or assent, using the appropriate consent or assent form, before proceeding.]

Section 1: Biography

- Can you tell me a little about yourself, for example your age, whether you are currently in school, and what life is generally like for you here in Buruburu?

Section 2: Cultural Barriers to SRHR Access

- When you think about sexual and reproductive health services in this community, what comes to mind first? (Probe, only if needed after the participant has answered in their own words: clinics, counselling, information sessions.)
- What, if anything, makes it difficult for girls and young women your age to access these services? (Probe: Are there specific beliefs, traditions, or expectations in the community that make this harder?)
- Sometimes one difficulty can make another one worse, for example, if not being able to afford a service also makes it harder to keep it private from family. Can you think of a time this happened, either to you or to someone you know?

Section 3: Sources of Support and Trust

- When you or girls you know need information or help related to SRHR, where do you usually go first? (Probe: friends, family members, religious leaders, clinics, community health workers, social media, teachers.)
- How much do you trust the source(s) you just mentioned, and why?

Section 4: Social, Familial and Physical Risks

- What do you think would happen, within your family or community, if people found out a girl was seeking SRHR services or information? (Probe: social risks,

family reaction, concerns about physical safety; and whether the reaction might differ depending on the type of service being sought, for example contraception versus HIV testing versus general counselling.)

- Have you, or someone you know, ever faced a difficult situation after seeking this kind of help? Could you tell me about it, only if you feel comfortable doing so?

Section 5: Comparative Reflection and Closing

- Have you heard whether things are similar or different for girls in other parts of Nairobi or other communities?
- Is there anything else about this topic that you think is important for us to know, that we have not already discussed?

Closing script: “Thank you so much for sharing your experience with me today. If anything we discussed has brought up difficult feelings, here is the contact information for [name of local support service], who you can reach for support. Do you have any questions for me before we finish?”

B. Focus Group Discussion (FGD) Guide (6 participants)

Ground rules (read aloud): “Thank you all for joining today. This discussion is about the experiences girls and young women in this community have around sexual and reproductive health services. There are no right or wrong answers. Please respect each other’s privacy; what is shared here should stay here. You do not have to share anything you are not comfortable with.”

Section 1: General Perceptions

- In general, what do girls and young women in this community think about seeking SRHR services?

Section 2: Cultural Barriers to SRHR Access

- What cultural barriers or community attitudes make it harder for girls to access these services?

Section 3: Sources of Support and Trust

- Where do girls in this community usually go for SRHR information or help?

Section 4: Social, Familial and Physical Risks

- What do people in this community generally think happens to a girl if others find out she sought SRHR services? (Probe: does this differ depending on the type of service being sought, for example contraception versus HIV testing versus general counselling?)

Section 5: Comparative Reflection and Closing

- Have you heard whether the situation is similar or different in other communities?
- Is there anything else the group feels is important to add?

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